NCE EXAM PREP

Practice Exam 15

Domain 3

Treatment Planning & Continuity of Care

26 Practice Questions | 34 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 03. Use the suggested 34-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

> A high school senior receiving school-based counseling for attention and organization difficulties will attend college in another city. The student wants to use campus counseling and disability supports but has never requested accommodations independently.

Which transition activity best promotes continuity and self-advocacy?

A) Ask the parent to arrange all college services so that support is in place before classes begin

B) Help the student identify documentation and access steps, practice making contact, and confirm a backup plan

C) Wait until academic problems occur because college services should respond only to demonstrated failure

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Question 2

> A 12-year-old in foster care wants help maintaining a homework and calming routine. A placement change is possible, and the current foster parent participates in planning. The child already uses a portable notebook successfully at school.

Which plan is most likely to remain useful if the placement changes?

A) Base the routine entirely on the current household's rules and reward system

B) Build portable notebook cues and child-owned coping steps, with caregiver supports that can be explained to a new placement

C) Delay routine planning until the child's long-term placement has been finalized

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Question 3

Which finding most directly indicates that a planned counseling approach should be deferred or modified?

A) The client expects the approach to require effort and temporary discomfort

B) The client asks to begin at a slower pace than the counselor usually recommends

C) Current conditions make the approach likely to cause harm or require abilities the client does not yet have

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Question 4

> A 17-year-old in a youth community program will turn 18 and transfer to the agency's adult service next month. The client wants a parent involved only in appointment reminders and is ready to make other care decisions independently. Agency policy requires new adult consent and releases at age 18.

Which transition plan best follows the client's preferences and the stated policy?

A) Continue the youth-service permissions automatically until the parent requests a change

B) Exclude the parent from all communication because adult services never involve family members

C) Complete the adult consent process, document the limited reminder role, and confirm the first adult-service appointment

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Question 5

> Assessment by an integrated care team identifies a client's first episode of psychosis. The client wants to remain in school and agrees to referral to a local coordinated specialty care program that provides counseling, medication services, family education, and supported education.

What is the best continuity step for the current counselor?

A) End counseling after giving the program's telephone number because specialty care now owns the case

B) Refer only for medication because educational goals should remain outside the specialty program

C) Facilitate the program intake and maintain an agreed bridge until the receiving team confirms care

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Question 6

> A client with chronic pain entered counseling to improve sleep and participation in family activities. After eight weeks, pain ratings are unchanged, but the client sleeps an additional hour most nights, attends family dinners again, and describes life as more manageable.

How should the counselor interpret these findings at the progress review?

A) Treatment is ineffective because the primary physical symptom has not decreased

B) Treatment is complete because improved functioning makes further review unnecessary

C) Treatment is producing meaningful functional gains, and the client should help decide whether the remaining plan needs revision

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Question 7

Which feature best demonstrates internal consistency in a treatment plan?

A) Each presenting problem is assigned the same number of objectives and interventions

B) Each intervention is linked to an objective and supported by the case conceptualization

C) Every objective uses the same review date regardless of urgency or complexity

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Question 8

> A client who communicates primarily in Vietnamese arrives for a planned session, but the qualified interpreter cancels unexpectedly. No qualified replacement is available that day, the client declines to proceed without one, and there is no acute safety concern.

Which response best supports continuity?

A) Ask the client's adult child to interpret so that the scheduled treatment plan remains on time

B) Reschedule promptly with a qualified interpreter and confirm how the client can access support before then

C) Conduct the session in English and document that the interpreter's absence was beyond the counselor's control

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Question 9

> A client attends counseling at a partner's request because of frequent cannabis use. The client does not currently want abstinence but is willing to track use and discuss how it affects sleep, spending, and the relationship.

Which initial objective best matches the client's current readiness?

A) Record use and related effects for three weeks, then review the pattern and readiness for change

B) Stop cannabis immediately and ask the partner to report any use before the next session

C) Avoid all discussion of cannabis until the client independently requests a reduction goal

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Question 10

> A client returns to outpatient counseling after a brief psychiatric hospitalization. The discharge plan includes a new safety plan, changed medication follow-up, and an appointment with a prescriber in three days. The client authorizes coordination, and the outpatient treatment plan has not yet been updated.

What should the counselor do to reestablish continuity?

A) Resume the previous outpatient plan until the prescriber determines whether the hospitalization changes counseling needs

B) Review the discharge information with the client, coordinate authorized follow-up, and reconcile the outpatient plan

C) Replace the outpatient goals with every hospital recommendation without discussing the changes with the client

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Question 11

A client treated for depression has lower symptom scores after ten weeks but has begun missing more work and reports that morning routines remain unmanageable. The client values employment stability more than further symptom reduction.

What is the best response to the mixed outcome pattern?

A) Review what produced symptom change and revise the plan to address the unresolved functional goal

B) Continue the plan unchanged because symptom reduction is the most objective indicator of success

C) End treatment because improvement in one outcome means the intervention has been effective overall

Question 12

A middle school student has begun refusing school. Interviews show that anxiety rises mainly on the bus, where peers have repeatedly harassed the student. The student wants to return, and the school can provide alternate transportation while addressing the harassment.

Which initial plan best integrates the available information?

A) Require use of the same bus so avoidance does not reinforce the student's anxiety

B) Focus exclusively on relaxation because school transportation is outside counseling goals

C) Pair a supported return plan with alternate transportation and coping objectives while the school addresses harassment

Question 13

A family has maintained stable housing for six months with help from a wraparound team. The family wants fewer formal services but still needs occasional help navigating benefits. A relative and a neighborhood organization can provide agreed support.

Which step-down plan best supports sustainable continuity?

A) Reduce services in stages, transfer specific tasks to chosen supports, and define indicators for renewed formal help

B) End all team services at once because stable housing shows that wraparound care is no longer needed

C) Keep every provider involved at the same intensity to prevent the possibility of future housing problems

Question 14

An integrated care client with irritable bowel syndrome and anxiety avoids eating before leaving home, worries about urgent symptoms, and has become socially isolated. The medical team has ruled out an acute condition, and the client wants to resume shared meals and community activities.

Which plan best reflects a biopsychosocial conceptualization?

A) Focus only on anxious thoughts because medical evaluation found no acute disease

B) Defer counseling until gastrointestinal symptoms disappear through medical treatment

C) Coordinate relevant medical care while targeting avoidance, coping, nutrition routines, and gradual return to valued activities

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Question 15

A graduate student seeks counseling for perfectionism that delays completion of a thesis. Both a cognitive-behavioral plan and an acceptance-based plan could address the problem. The student prefers an acceptance-based approach and wants progress measured by submitting drafts on schedule.

Which planning decision is most appropriate?

A) Select the cognitive-behavioral plan because counselor preference should resolve choices between suitable approaches

B) Combine every technique from both approaches so that no potentially helpful intervention is omitted

C) Develop the acceptable evidence-informed plan around scheduled draft submission and monitor whether it produces progress

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Question 16

What does attainable mean when evaluating a treatment objective?

A) The objective is realistically achievable from the client's baseline with available time and supports

B) The objective can be completed without discomfort, setbacks, or assistance from other people

C) The objective reflects the counselor's ideal outcome even if the client doubts it can be reached

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Question 17

A 14-year-old is frequently late to first period after staying online past the agreed bedtime. The adolescent helps create a plan, values weekend recreation with family, and can place the device in a common charging area. The caregiver can provide one consistent morning cue.

Which behavioral plan is most appropriate?

A) Define the charging and morning routines, reinforce on-time days with agreed weekend recreation, and track arrival time

B) Remove all device access for an unspecified period after each late arrival and allow the caregiver to change the consequence

C) Ask the adolescent to promise better choices and discuss lateness only when a school complaint occurs

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Question 18

> A Muslim client explains that fasting during Ramadan, late evening family meals, and early prayer will temporarily change sleep and activity patterns. The client wants to continue counseling for depression and asks that the plan reflect these personally meaningful practices.

How should the counselor adapt the plan?

A) Collaborate on realistic sleep, activity, and session objectives that fit the client's stated observance and preferences

B) Treat fasting as a depressive symptom because it changes the client's usual eating and sleeping schedule

C) Keep all objectives unchanged because cultural and religious practices should not affect clinical planning

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Question 19

> A client seeking help for insomnia works rotating day and night shifts that cannot be changed. The initial objective requires the same bedtime and wake time every day, and the client has been unable to follow it despite using other sleep strategies consistently.

What is the most appropriate plan revision?

A) Develop shift-specific sleep routines and a feasible anchor period while continuing to monitor sleep outcomes

B) Retain the fixed schedule because modifying an evidence-informed plan would reward nonadherence

C) End insomnia work until the client obtains employment with a stable schedule

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Question 20

> A client completing counseling for anger outbursts has an upcoming co-parenting meeting, a situation that previously led to escalation. The client can recognize jaw tension as an early cue and has successfully used planned pauses during recent disagreements.

Which maintenance plan best prepares the client for the meeting?

A) Avoid all direct co-parenting meetings indefinitely so that anger cannot recur

B) Depend on motivation to remain calm and contact the counselor only if another outburst occurs

C) Link the early cue to a rehearsed pause, identify support after the meeting, and define when to seek follow-up

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Question 21

Which element is most characteristic of an aftercare plan?

A) A detailed reconstruction of every topic discussed during active treatment

B) Specific post-treatment supports, responsibilities, and contingencies for maintaining gains

C) A statement that the client may contact any provider if a future problem develops

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Question 22

> The parent of a child receiving counseling reports untreated panic symptoms and asks the child's counselor to provide therapy for the parent during the child's sessions. The parent is willing to seek a separate provider, and the child's progress depends partly on the parent's ability to follow the home plan.

Which continuity response is most appropriate?

A) Treat the parent's panic within the child's sessions because caregiver functioning affects the child

B) Tell the parent to find care independently so the child's counseling remains completely separate

C) Provide an appropriate adult referral and, with permission, coordinate only the caregiver support relevant to the child's plan

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Question 23

> A client with panic disorder and a seizure disorder is considering interoceptive exposure exercises that deliberately produce dizziness and rapid breathing. The counselor has no information about whether those exercises are medically appropriate for this client.

How should the counselor address this issue in the plan?

A) Begin the standard exercises because temporary physical sensations are the intended part of treatment

B) Coordinate appropriate medical input and use suitable plan components while clarifying which exercises are safe

C) Remove all exposure-based goals permanently because any seizure history rules out behavioral treatment

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Question 24

> A client in career counseling met an objective of submitting four job applications weekly ahead of schedule. The broader employment goal remains active, and the client now reports anxiety about interviewing.

How should the plan be revised?

A) Continue the application objective unchanged until the original review date to preserve consistency

B) Document the gain and collaboratively add an interview-preparation objective while maintaining useful application habits

C) Close the employment goal because completing one objective demonstrates readiness to manage the remaining steps alone

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Question 25

> A client with no acute safety concern says rising out-of-pocket costs require ending counseling earlier than planned. The client has made partial progress and wants to preserve gains but cannot continue at the current fee.

What is the most appropriate response?

A) Review progress and remaining needs, create a focused ending plan, and offer affordable continuation or reentry options

B) Insist that the client complete the original course because early termination may weaken treatment gains

C) End immediately without further planning because financial decisions fall outside the counseling relationship

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Question 26

> A university conduct program requires a student to attend six counseling sessions addressing consequences of alcohol use. The student does not agree that alcohol is the main problem but wants help improving grades after several missed classes. The program permits individualized goals in addition to the required focus.

Which initial plan best balances the mandate and collaboration?

A) Explain the required focus and confidentiality limits, then link alcohol-related review with the student's academic goal

B) Use all six sessions only for alcohol education because required services cannot include client-selected goals

C) Avoid discussing alcohol until the student independently identifies it as a problem

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PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: B

Correct Answer: Help the student identify documentation and access steps, practice making contact, and confirm a backup plan

Explanation: Practicing the access process supports the student's developmental move toward independent self-advocacy while still providing structure. Having a parent manage every step or waiting for failure would not prepare the student for the transition.

Question 2 — Correct: B

Correct Answer: Build portable notebook cues and child-owned coping steps, with caregiver supports that can be explained to a new placement

Explanation: Portable tools and child-owned skills can remain stable even if the caregiving setting changes. A plan dependent entirely on one household may not transfer, while postponement would withhold useful support during uncertainty.

Question 3 — Correct: C

Correct Answer: Current conditions make the approach likely to cause harm or require abilities the client does not yet have

Explanation: A contraindication is based on a meaningful risk or prerequisite that affects suitability, not simply on anticipated effort. A request for pacing can often be incorporated through collaborative adaptation rather than treated as a reason to reject the approach.

Question 4 — Correct: C

Correct Answer: Complete the adult consent process, document the limited reminder role, and confirm the first adult-service appointment

Explanation: The stated policy requires new adult permissions, and the client's preference allows a specific, limited role for the parent. Automatically carrying forward youth permissions or excluding all family participation would override either policy or client choice.

Question 5 — Correct: C

Correct Answer: Facilitate the program intake and maintain an agreed bridge until the receiving team confirms care

Explanation: Coordinated specialty care matches the client's clinical and educational needs, but a referral does not transfer responsibility until connection is confirmed. A brief bridge reduces the risk of losing care during intake.

Question 6 — Correct: C

Correct Answer: Treatment is producing meaningful functional gains, and the client should help decide whether the remaining plan needs revision

Explanation: The plan targeted sleep and participation as well as the experience of pain, and those functional outcomes have improved. Unchanged pain ratings should be discussed rather than treated as proof of total failure or ignored because other gains occurred.

Question 7 — Correct: B

Correct Answer: Each intervention is linked to an objective and supported by the case conceptualization

Explanation: Internal consistency means that the plan's actions have a clear rationale and are directed toward identified outcomes. Equal numbers of entries or identical deadlines may look orderly but do not demonstrate clinical alignment.

Question 8 — Correct: B

Correct Answer: Reschedule promptly with a qualified interpreter and confirm how the client can access support before then

Explanation: The client has declined to proceed without the communication access needed for meaningful participation, and there is no fact requiring an emergency exception. Prompt rescheduling and an interim access plan preserve continuity without relying on an unqualified family interpreter.

Question 9 — Correct: A

Correct Answer: Record use and related effects for three weeks, then review the pattern and readiness for change

Explanation: The client is willing to observe consequences but has not chosen abstinence, making monitoring and collaborative review a realistic initial objective. Imposed abstinence exceeds current readiness, while avoiding the topic would fail to use the client's expressed willingness.

Question 10 — Correct: B

Correct Answer: Review the discharge information with the client, coordinate authorized follow-up, and reconcile the outpatient plan

Explanation: Hospitalization has introduced new safety, medication, and follow-up information that should be integrated collaboratively. Simply resuming the old plan ignores important changes, while adopting every recommendation without discussion removes the client from planning.

Question 11 — Correct: A

Correct Answer: Review what produced symptom change and revise the plan to address the unresolved functional goal

Explanation: Lower symptom scores and worsening work functioning form a mixed result that should guide targeted revision. Declaring overall success or continuing unchanged would disregard the client's valued outcome and the functional data.

Question 12 — Correct: C

Correct Answer: Pair a supported return plan with alternate transportation and coping objectives while the school addresses harassment

Explanation: The school refusal is connected to a real environmental threat as well as anxiety, so the plan should address both. Requiring immediate use of the same bus could expose the student to continued harassment, while relaxation alone would omit the relevant context.

Question 13 — Correct: A

Correct Answer: Reduce services in stages, transfer specific tasks to chosen supports, and define indicators for renewed formal help

Explanation: Gradual de-intensification lets the family assume more ownership while preserving clearly defined support and reentry pathways. Abrupt withdrawal risks a gap, whereas unchanged service intensity disregards the family's gains and preference for greater independence.

Question 14 — Correct: C

Correct Answer: Coordinate relevant medical care while targeting avoidance, coping, nutrition routines, and gradual return to valued activities

Explanation: The case includes interacting physical, behavioral, emotional, and social factors, all of which can inform the plan. Treating the symptoms as exclusively psychological or delaying counseling until all gastrointestinal symptoms resolve would oversimplify the formulation.

Question 15 — Correct: C

Correct Answer: Develop the acceptable evidence-informed plan around scheduled draft submission and monitor whether it produces progress

Explanation: Both approaches are described as suitable, so the client's informed preference and measurable functional outcome can guide selection. Combining all techniques may dilute coherence, and counselor preference alone is not a sufficient reason to reject the client's choice.

Question 16 — Correct: A

Correct Answer: The objective is realistically achievable from the client's baseline with available time and supports

Explanation: An attainable objective may still be challenging, but it is feasible in light of the client's starting point, resources, and time. It does not need to be effortless, and counselor preference cannot substitute for realistic client capacity.

Question 17 — Correct: A

Correct Answer: Define the charging and morning routines, reinforce on-time days with agreed weekend recreation, and track arrival time

Explanation: The plan specifies antecedent routines, a client-valued reinforcer, and an observable outcome that can be reviewed. Unspecified punishment and occasional discussion do not create consistent contingencies or usable progress data.

Question 18 — Correct: A

Correct Answer: Collaborate on realistic sleep, activity, and session objectives that fit the client's stated observance and preferences

Explanation: The client has identified a temporary, meaningful context that changes what is feasible. Adapting objectives preserves treatment direction without pathologizing observance or pretending that scheduling constraints are irrelevant.

Question 19 — Correct: A

Correct Answer: Develop shift-specific sleep routines and a feasible anchor period while continuing to monitor sleep outcomes

Explanation: The fixed schedule conflicts with an employment condition the client cannot change, despite consistent effort with other strategies. A feasible adaptation allows the counselor to test outcomes without labeling a structural barrier as nonadherence.

Question 20 — Correct: C

Correct Answer: Link the early cue to a rehearsed pause, identify support after the meeting, and define when to seek follow-up

Explanation: The plan builds on a personally recognized warning sign and a skill the client has already used successfully. Indefinite avoidance prevents practice of gains, while motivation without a specific response leaves the known trigger insufficiently addressed.

Question 21 — Correct: B

Correct Answer: Specific post-treatment supports, responsibilities, and contingencies for maintaining gains

Explanation: Aftercare is future-focused and identifies how gains will be supported after active treatment ends. A historical reconstruction or a vague invitation to contact any provider does not assign concrete actions or contingencies.

Question 22 — Correct: C

Correct Answer: Provide an appropriate adult referral and, with permission, coordinate only the caregiver support relevant to the child's plan

Explanation: The parent's needs warrant their own provider rather than being treated within the child's sessions. Limited, authorized coordination can still support implementation of the child's home plan without making the parent a second client in that treatment.

Question 23 — Correct: B

Correct Answer: Coordinate appropriate medical input and use suitable plan components while clarifying which exercises are safe

Explanation: Deliberately provoking physical sensations warrants clarification because the client's seizure disorder may affect exercise selection. The counselor need not assume either that every standard exercise is safe or that all behavioral treatment is permanently contraindicated.

Question 24 — Correct: B

Correct Answer: Document the gain and collaboratively add an interview-preparation objective while maintaining useful application habits

Explanation: Meeting an objective early is information for progression, not a reason to retain a completed task or close the broader goal. The next objective should address the newly identified barrier while preserving behaviors that remain useful.

Question 25 — Correct: A

Correct Answer: Review progress and remaining needs, create a focused ending plan, and offer affordable continuation or reentry options

Explanation: The client has the right to end care, and the financial constraint can be addressed through a planned, clinically informed transition. Coercing completion or ending without review would fail to support maintenance of partial gains.

Question 26 — Correct: A

Correct Answer: Explain the required focus and confidentiality limits, then link alcohol-related review with the student's academic goal

Explanation: The counselor should be transparent about the mandated elements while using the flexibility allowed by the program to build client-relevant goals. Ignoring the required focus or excluding the student's academic concern would weaken informed collaboration.
